**Protocol for Treating Stress Urinary Incontinence (SUI) Using Low-Intensity Shockwaves**



**Device**: MoreNova - Li50F Linear Transducer
**Document Version**: 2.0
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## 1. Purpose and Scope

This protocol provides a standardized treatment plan for using low-intensity shockwaves to manage stress urinary incontinence (SUI). It is intended for healthcare providers using the MoreNova Device with Li50F Transducer to target the tissues surrounding the urethra and sphincter muscles.

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## 2. Introduction

Low-Intensity Extracorporeal Shockwave Therapy (LiESWT) is a non-invasive technology that uses acoustic waves to stimulate tissue regeneration, enhance blood circulation, and strengthen muscle function. It is clinically proven for treating stress urinary incontinence (SUI) by targeting weakened pelvic floor muscles and the urethral sphincter. For further information on clinical studies and efficacy, visit our Clinical Data Center.

Device Overview:
**MoreNova** is a portable Electromagnetic (EM) Low-Intensity Shockwave (LISW) device designed to reduce inflammation, dissolve calcifications, and stimulate angiogenesis, improving outcomes for various conditions in men and women.

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## 3. Background

**Condition Overview**:

- SUI is an involuntary leakage of urine due to increased abdominal pressure during activities like coughing, sneezing, or exercising.
- It often results from weakened pelvic floor muscles and urethral sphincter dysfunction.

**Mechanism of Action**:

- Low-intensity shockwave therapy stimulates neovascularization, enhances blood flow, and promotes tissue regeneration, improving pelvic floor strength and urethral function.

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## 4. Patient Selection Criteria

**Inclusion Criteria**:

- Women aged 20-75 years.
- Diagnosed with SUI by a gynecologist for at least 3 months.
- Persistent SUI symptoms despite conservative management.

**Exclusion Criteria**:

- History of surgery in the vulvar or vestibular region.
- Blood coagulation disorders.
- Active pelvic inflammatory conditions.
- Vaginal infections (e.g., yeast infection or bacterial vaginosis).
- Pregnancy or lactation.
- History of pelvic or external genitalia irradiation within the past year.
- Chronic neurological disorders cause localized pain.
- Presence of an electromagnetic-sensitive medical device.

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**5. Pre-Treatment Screening**

To determine the patient's suitability for LiESWT, conduct the following:

- Comprehensive medical history review.
- Physical examination to assess pelvic floor condition.
- Diagnostic tests: Urinalysis to rule out infections, and pelvic ultrasound for anatomical assessment, if indicated.

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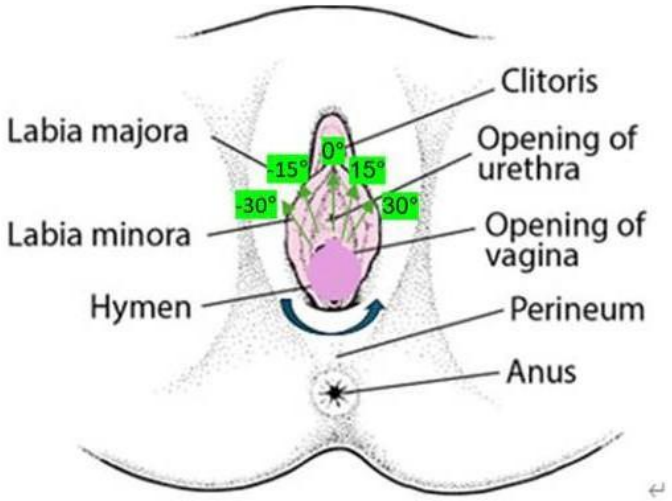
**6. Treatment Protocol**

**Preparation**:

- Conduct a thorough medical history and gynecological examination.
- Counsel the patient about the procedure, benefits, risks, and expected outcomes.

**Procedure**:

- **Number of Sessions**: 6-8 sessions based on severity.
- **Session Frequency**: Twice per week.
- **Session Duration**: 20 minutes. (angled rotation beneath the urethra)
- **Applications Per Session**: 5.




**The rotation pattern**:

1. -30°
2. -15°
3. 0° (beneath the urethra)
4. 15°
5. 30°

- **Shocks Per Application**: 500.
- **Total Shocks Per Session**: 2500.

**Treatment Parameters**:

- **Energy (HV) Settings**: 2.0 kV.
- **Frequency (FRQ)**: 2.0 Hz.

**Post-Treatment Care**:

- Advise patients to avoid strenuous physical activity for 24 hours.
- Recommend hydration and report any adverse events.

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### 7. Treatment Areas and Techniques

The treatment focuses on the following anatomical areas:

- Pelvic floor muscles.
- Urethral sphincter.

Ensure correct Transducer placement beneath the urethra and apply gentle pressure. Rotate the Transducer in 15° increments to cover all necessary areas, following the predefined sequence.

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### 8. Patient Comfort and Safety

To ensure patient comfort and safety during the procedure:

- Position the patient in a supine or lithotomy position for optimal access.
- Use conductive gel for effective shockwave transmission and minimal discomfort.
- Continuously monitor patient feedback during the session and adjust settings if needed.

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### 9. Treatment Outcomes and Follow-Up

**Expected Results**:
Patients typically experience symptom improvement within 3–6 weeks of starting treatment. Regular assessments are recommended to evaluate progress.

**Follow-Up Visits**:
Schedule bi-weekly follow-ups to monitor patient outcomes and adjust the treatment plan as needed.

**Long-Term Maintenance**:
For sustained improvement, consider maintenance sessions every 3–6 months based on individual needs.

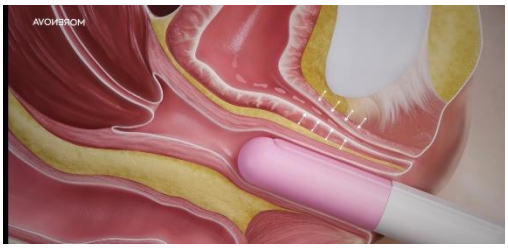
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**10. Visuals and Diagrams**

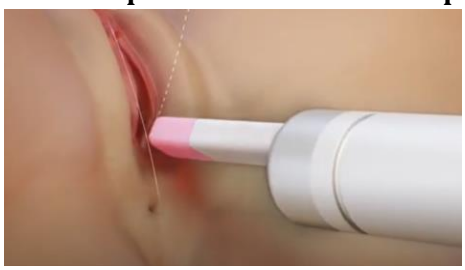
**Li50F**



- **Anatomical areas being treated.**



- **Rotation pattern for Transducer placement.**



## 11. Quality Control

**Device Calibration**:

- Verify device settings before each session.
- The MoreNova Device recognizes preprogrammed parameters, ensuring correct settings are automatically applied.

**Clinician Training**:

- Ensure all operators are thoroughly trained in the use of the MoreNova Device and Li50F Transducer.
- Clinicians should be familiar with this protocol and review the "Instructions for Use" provided by the manufacturer.

**Checklist for Each Session**:

- Confirm that the patient meets the inclusion criteria and has no contraindications.
- Ensure the device and Transducer are clean, sanitized, and function correctly.
- Confirm that the Transducer placement aligns with the anatomical areas specified in the protocol.

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## 12. References

For clinical studies and additional resources, visit the Initia Clinical Data Center.